

Patient Name	: Mr.SARAVANAN SADASIVAN	Collected	: 06/Nov/2025 06:41AM
Age/Gender	: 45 Y 3 M 8 D /M	Received	: 06/Nov/2025 01:51PM
UHID/MR No	: APJ1.0022205555	Reported	: 06/Nov/2025 02:31PM
Visit ID	: DNSWOPV1524	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: PUP 24X7_CREDIT
IP/OP NO	:	Center location	: Nelakadiranahalli,Bangalore

DEPARTMENT OF BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	180	mg/dL	< 200	CHOD-PAD
TRIGLYCERIDES	225	mg/dL	< 150	GPO-PAP
HDL CHOLESTEROL	42	mg/dL	>=40 Desirable	Enzymatic Immunoinhibition
NON-HDL CHOLESTEROL	138	mg/dL	<130	Calculated
LDL CHOLESTEROL	92.9	mg/dL	<100	Calculated (Friedewald)
VLDL CHOLESTEROL	45	mg/dL	<30	Calculated
CHOL / HDL RATIO	4.28		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	0.37		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: Low risk – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.

Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events.

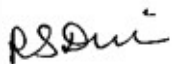
(Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.

2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing.

The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

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SIN No:BI28796223

This test has been performed at Apollo Health and Lifestyle ltd- RRL Bangalore , Koramangala.

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DEPARTMENT OF BIOCHEMISTRY

APOLLO VITAMIN CHECK - BASIC

Test Name	Result	Unit	Bio. Ref. Interval	Method
CALCIUM , SERUM	9.74	mg/dL	8.6-10	NM-Bapta



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DEPARTMENT OF IMMUNOLOGY

APOLLO VITAMIN CHECK - BASIC

Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN D (25 - OH VITAMIN D) , SERUM	57.9	ng/mL	30-100	ECLIA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

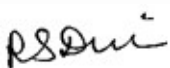
Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements.

Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:- Inadequate exposure to sunlight, Dietary deficiency, Vitamin D malabsorption, Severe Hepatocellular disease., Drugs like Anticonvulsants, Nephrotic syndrome.

Increased levels:- Vitamin D intoxication.

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SIN No:IM10781083

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DEPARTMENT OF IMMUNOLOGY

APOLLO VITAMIN CHECK - BASIC

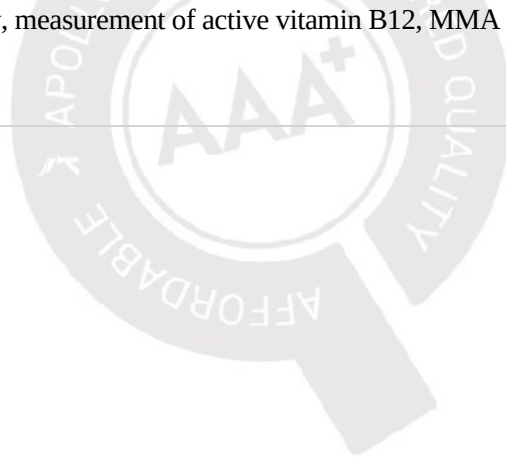
Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN B12 , SERUM	625	pg/mL	197-771	ECLIA

Comment:

Population based data reflecting exact scenario of vitamin B12 levels in Indian population is still evolving, however, different studies reporting a deficiency in adults, pregnant women and children ranging from 16% to 77% with average of about 47%. This high incidence is attributed to vegetarian food habits of large majority of Indian population.

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. B12 levels in the range of 150 to 190 pg/ml may not be associated with any clinical manifestations, while B12 levels below 100 pg/ml are often associated with clinical symptoms. However, for an individual based on other co-morbid conditions or other nutritional deficiency (especially folate) the manifestations can vary accordingly.

If clinical symptoms suggest deficiency, measurement of active vitamin B12, MMA and homocysteine should be considered as further workup.



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DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Interval	Method
TESTOSTERONE, TOTAL , SERUM	344	ng/dL	249-836	ECLIA

Comment:

Testosterone exhibits significant circadian variations in young men, and early morning samples are recommended. Increased levels are seen in precocious puberty (males), androgen resistance, CAH, ovarian stromal hyperthecosis. Decreased levels are seen in delayed puberty (males), gonadotropin deficiency, testicular feminization, estrogen therapy and certain systemic diseases.

Reference Values for Females (8 - 18 years) Characterized by Tanner Stage:

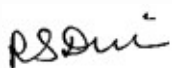
Tanner Stage	Lower Limit (ng/dL)	Upper Limit (ng/dL)
Stage 1	< 2.50	6.10
Stage 2	< 2.50	10.40
Stage 3	< 2.50	23.70
Stage 4	< 2.50	26.80
Stage 5	4.60	38.30

Reference Values for Males (7 - 18 years) Characterized by Tanner Stage:

Tanner Stage	Lower Limit (ng/dL)	Upper Limit (ng/dL)
Stage 1	< 2.50	2.50
Stage 2	< 2.50	432.0
Stage 3	64.90	778.0
Stage 4	180.0	763.0
Stage 5	188.0	882.0

*** End Of Report ***

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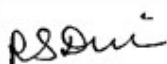


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8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only



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Apollo Health and Lifestyle Limited

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